30 October 2026 | Evaluation plan v12 | Discussion draft
MEMORANDUM

# Neocam: consistent investment decisions from existing evidence

## Action required

Estimate the value of neonatal video support compared with telephone consultation using existing evidence and transparent assumptions. Align decisions, actual model inputs, equipment ownership, budget schedules and current reporting. No new recruitment or structured elicitation is required for this preliminary evaluation.

| | |
|---|---|
| **Author** | Prepared for Dr Dylan Mordaunt, Chief Medical Officer |
| **Presented by** | Neonatal, retrieval, virtual-care and finance partners |

## Executive summary

1. Decisions. Compare telephone, mobile teleconference, Panda Vision, ceiling/wall PTZ and a complete tablet/OBSBOT kit. Separate new investment, retention and shared infrastructure. The kit is a conceptual comparator, not approved clinical equipment.

2. Populations. Use pilot (3 sites), CHHHS hospital/emergency network (10), CHHHS + TCHHS (14) and illustrative Queensland (33). Historical births and missingness remain explicit; these are not certified deployment inventories.

3. Model. Retain immediate transfer, delayed rescue and final local care; failed video falls back to telephone. Consultation exists in both arms. Keep incremental specialist time, paid capacity and hospital resource prototypes separate.

4. Configuration and ownership. One validated configuration supplies the executed parameters and pathway assumptions. One ownership table feeds retained capital and replacement budgets. Year-1 CHHHS new purchases: 7 installations, $70,000. Configured Panda benchmark: $10,000 per installation; verify invoices. Remaining life is a budget assumption.

5. Perspectives and uncertainty. Reconcile health-system, bounded societal, provider, patient/family and conditional state/Commonwealth views. Report paired probabilistic structures, clinical plausibility, null/sceptical effects, and value of perspective. Funding and subsidies do not create social resources.

6. Health outcomes. Do not infer mortality or QALY gains from transport changes. The illustrative utility scenarios begin at the source age of 11 and are not included in primary results. Additional survival with morbidity is not excluded.

7. Evidence and decisions. Replace assumptions when ordinary invoices or routine records are available; complete critical full texts and independent clinical/economic review before publication. Assess safe care and equity alongside cost, not transfer reduction alone.

| Engagement and consultation | |
|---|---|
| **Consumers** | Include family travel/time and disclose omitted caregiver, education, disability-service and environmental consequences. |
| **Clinicians and staff** | Simulated review is author self-review. Obtain independent clinical/economic review before claiming validation. |
| **External stakeholders** | Use current records where available; uncertain effect estimates remain explicit, not locally fitted. |

| Implications | |
|---|---|
| **Budget** | Annual cash purchases, opportunity costs and funding allocations are separate. No expenditure approval is implied. |
| **FTE** | Incremental hub workload depends on video time, not an arbitrary count of sites. Repeated contacts per infant remain an open workload assumption. |

| Implications | |
|---|---|
| **Risk** | Current outputs are regenerated and hash-bound. Historical tables and slides are not silently presented as the current analysis. |
| **Legal** | No patient-level records added. Corporate templates/source excerpts must remain private pending redistribution permission. |
| **ICT** | A passing ordinary test suite is not full assurance. Lock, Gitleaks, requested tools and hosted CI require actual execution. |
| **Impact on First People's Health** | Rural access, clinical appropriateness and disability-inclusive consequences remain relevant even where the service is not cost-saving. |

**Strategic Plan alignment**

Our care: appropriate neonatal support. Our people: explicit workload and usable systems. Equity: remote access. Sustainability: coherent investment and perspective accounting.

**Attachment/s**

Current CSV tables, plots, report and presentation share one resolved-input snapshot and release manifest. Source registers remain incomplete where originals/full texts were unavailable. This is not a validated cost-utility or procurement recommendation.